

Nixara

A recommendation tool. Nixara suggests the decision; you make it.

Every figure is worth checking before you act on it.

NIK·SA·RA /NIK'SCI:RƏ/ · from *nix* (clarity, light) + *ara* (direction) — illuminating the path forward in your data

Upload CSV / Excel · Connect Tableau or Power BI · Get executive-grade AI reports

- Dashboard
- Memory
- Inbox
- Outcomes
- FAQ

Ψ Good evening.

Decisions don't clock out — but you probably should. →

- Upload CSV / Excel
- Tableau
- Power BI

Drop a CSV or Excel file here, or click to browse
.csv, .xlsx, .xls

HEALTHCARE_DATASET.XLSX · 55500 ROWS

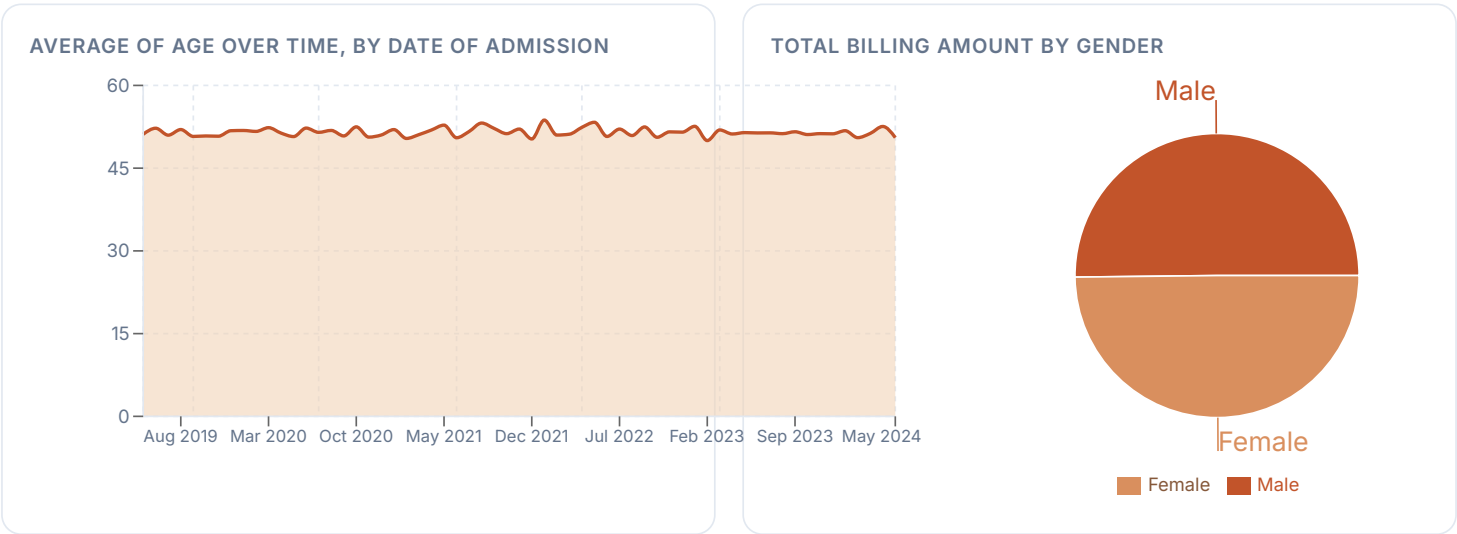
ROWS	COLUMNS	NUMERIC FIELDS	QUALITY SCORE
55,500	15	3	100

Nothing knocked this down

No blank-heavy or mixed-type columns, at least one measurable metric, and a workable number of rows and columns.

Name	Age	Gender	Blood Type	Medical Condition	Date of Admission	Doctor	Hospital
Bobby Jackson	30	Male	B-	Cancer	Jan 31, 2024	Matthew Smith	Sons and

Leslie Terry	62	Male	A+	Obesity	Aug 20, 2019	Samantha Davies	Kim Inc
Danny Smith	76	Female	A-	Obesity	Sep 22, 2022	Tiffany Mitchell	Cook PL
Andrew Watts	28	Female	O+	Diabetes	Nov 18, 2020	Kevin Wells	Hernand
Adrienne Bell	43	Female	AB+	Cancer	Sep 19, 2022	Kathleen Hanna	White-W
Emily Johnson	36	Male	A+	Asthma	Dec 20, 2023	Taylor Newton	Nunez-H
Edward Edwards	21	Female	AB-	Diabetes	Nov 3, 2020	Kelly Olson	Group M
Christina Martinez	20	Female	A+	Cancer	Dec 28, 2021	Suzanne Thomas	Powell R
Jasmine Aguilar	82	Male	AB+	Asthma	Jul 1, 2020	Daniel Ferguson	Sons Ric
Christopher Berg	58	Female	AB-	Cancer	May 23, 2021	Heather Day	Padilla-V



Name any of these in your question to chart it: **Gender** (2) · **Blood Type** (8) · **Medical Condition** (6) · **Insurance Provider** (5) · **Admission Type** (3) · **Medication** (5) **+1 more**

REPORT SETUP

Common decisions (optional starting point)

Pricing / discounting

Cost reduction

Expansion / hiring

Customer retention

Operational efficiency

Risk / compliance

Decision Context

We reprice next year. Is smoking or chronic disease the bigger cost driver, do they stack, and which plan carries more risk than it is priced for?

Role

CFO

Time Horizon

This quarter

OpenAI API Key (optional)

Not stored — cleared when you close or refresh this tab.

✓ Key active

Generate Reports

AI REPORTS

- Executive Summary
- Operational Detail
- Risk Report

Reading this report: 🔍 **source** traced to your data 🧮 **calculated** computed by Nixara from your data
unverified could not be matched, may still be right

Nixara checked this report against your file and changed 2 numbers

A first draft cited figures that matched nothing in your data. Nixara asked for a rewrite, and this is the result.

FIRST DRAFT SAID	WHERE	OUTCOME
\$477.00	Risks If You Wait	number removed, wording kept
\$232.00	Risks If You Wait	number removed, wording kept

Being unmatched does not make a figure wrong. It means Nixara could not prove it from your data, so it chose not to stand behind it.

SITUATION

Chronic disease appears to be a substantial cost driver, with conditions like diabetes and obesity each generating billing amounts over **\$238.00** million. The financial burden from smoking-related conditions, although significant, is not as prominently delineated in the current data. This suggests that while both are influential, chronic diseases pose a larger immediate cost driver.

WHAT THIS MEANS FOR YOU

As Chief Financial Officer, deciding on next year’s repricing requires focusing on the rising costs associated with chronic diseases. These conditions have a major impact on current financials and could risk inflating claims beyond current pricing structures if left unmanaged. Given these dynamics, it is essential to reassess which plans are more exposed to these risks and update them accordingly for sustained financial health.

RECOMMENDED ACTIONS

- ⛔

RESTRICT

Restrict policy coverage for chronic
- ✅

APPROVE

Approve a targeted increase in
- 📅

MANDATE

Mandate an annual review process

diseases by implementing stricter reimbursement terms within the next fiscal quarter.

premiums specifically for plans with high exposure to chronic condition claims, effective immediately.

for plans every quarter to systematically identify and adjust for risk factors related to chronic diseases.

⚠ RISKS IF YOU WAIT

⚠ Delay in appropriately repricing could accentuate claim costs due to chronic diseases, outpacing adjustments made by competitors and impacting competitiveness.

⚠ Costs attributable to chronic illnesses could exceed **\$237.00** million thresholds, further challenging the sustainability of plans if not adjusted timely this quarter.

↓ Download as Word

↓ Download as PDF

 Record Your Decision

What did you decide to do with these recommendations?

Which recommendation does this apply to? *

- ☐ All recommendations
- ☐ 1. Restrict policy coverage for chronic diseases by implementing stricter reimbursement terms within the next fiscal quarter.
- ☐ 2. Approve a targeted increase in premiums specifically for plans with high exposure to chronic condition claims, effective immediately.
- ☐ 3. Mandate an annual review process for plans every quarter to systematically identify and adjust for risk factors related to chronic diseases.

Who is responsible for this action? (optional)

e.g. Sarah Chen, CFO team, Operations dept...

Notes (optional)

Revisit end of Q3, pending budget approval...

Due by (optional — only applies if you approve)

10/09/2026

 **Approve**

 **Reject**

⏸ Postpone